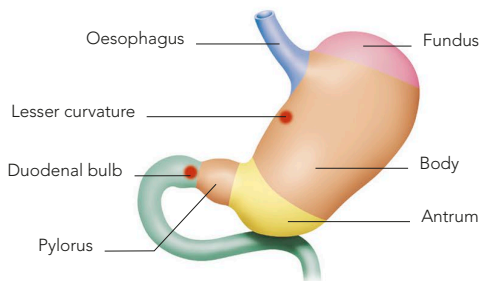


# DIGESTIVE TRACT DISORDERS

LIFESTYLE FACTORS, SUCH AS EXCESSIVE ALCOHOL CONSUMPTION, A POOR OR LOW-FIBRE DIET, AND FOOD SENSITIVITIES, CONTRIBUTE TO MANY DIGESTIVE TRACT DISORDERS, ALTHOUGH SOME PROBLEMS ARE RELATED TO BACTERIA OR TO A COMPROMISED IMMUNE SYSTEM.

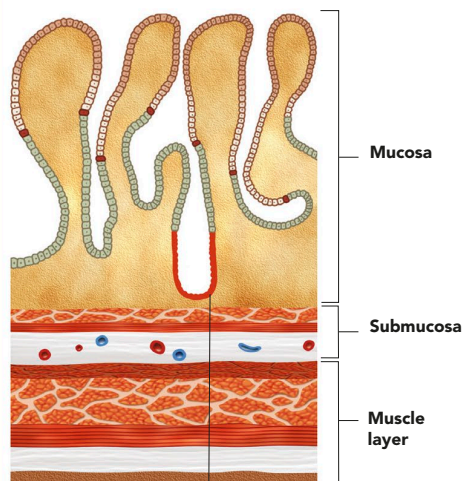
## PEPTIC ULCERS

Most peptic ulcers are associated with *Helicobacter pylori* bacteria. These damage the mucous lining that normally protects against the powerful acidic juices in the stomach and first part of the duodenum. Other contributory factors include alcohol consumption, smoking, certain medications, family history, and diet. Upper abdominal pain is a common symptom. With a duodenal ulcer, this is often worse before a meal and relieved by eating; in a gastric ulcer, eating tends to aggravate the pain.



### SITES OF PEPTIC ULCERS

A common site for ulcers is in the first part of the duodenum (duodenal bulb). In the stomach, most ulcers develop in the lesser curvature.

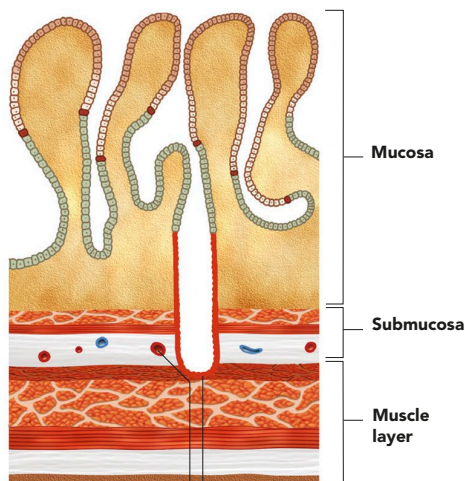


#### Erosion

In the early stages the lining is only partly destroyed, producing a shallow area of damage

### EARLY ULCER

If the protective mucus barrier coating the stomach lining breaks down, gastric juices containing strong acid and enzymes come into contact with mucosal cells.



#### Blood vessel

Bleeding (haemorrhage) can result if a vessel is breached by a deepening ulcer

#### Deepening ulcer penetrates muscle layer

### PROGRESSIVE ULCERATION

A true ulcer penetrates the entire lining (mucosal layer) as well as the submucosa and muscle layers. In severe cases, it can perforate the stomach or duodenal wall.